
Extending the reach of specialist paediatric expertise *across* *Queensland*

Strategic options and recommendation for Children's Health Queensland
(CHQ)

How can CHQ extend the *reach* of its specialist expertise across Queensland while protecting the *concentration* of capability that quaternary excellence needs?

Three gaps define CHQ's *strategic challenge*

01 • CAPABILITY GAP

37%

of Category 2 outpatients are seen on time.

Demand growing 2.9–5.1% a year against 1.2% population growth. Vacancy rate 14.2%; staff engagement 68%.

02 • GEOGRAPHIC GAP

80%

of Greater Brisbane's extra children will live outside QCH's catchment.

125,953 of 156,990. Growth corridors such as Ipswich (+3.2% a year) sit far from the specialist hub.

03 • OPERATING-MODEL GAP

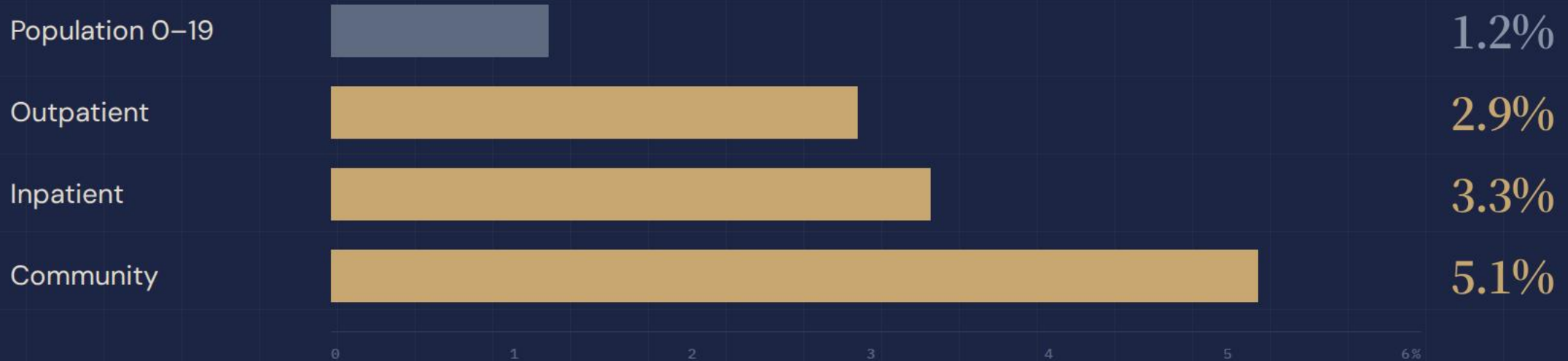
0 of 7

networked-care KPIs reported in 2024–25.

The statewide role is not mandated, funded or governed. Operating surplus fell from \$10.99m to \$0.13m.

Demand is growing *two to four times* faster than the children CHQ serves

PROJECTED GROWTH, % A YEAR (MASTER PLAN)



Scarce specialist expertise is both CHQ's *advantage* and its *constraint*

PESTEL

Policy and demographics push care *closer to home*.

Five Forces

Supplier power high (scarce specialists); buyer power high (CHQ cannot direct HHSs); rivalry and entrants low.

VRIN

Quaternary expertise is a sustained advantage, but the same people carry care, training and statewide support.

Budget

Expenses +8.5% vs income +7.4% in 2024–25.

THE INSIGHT WE CARRY FORWARD

Centralisation creates capability *depth*.
Networking creates capability *reach*.

Three options, each a *different degree* of change

OPTION 1

Extension of the existing path

Extend networked care

Scale advice, outreach, shared care and local follow-up in 2–3 pathways with willing HHS partners.

RISK • stays fragmented if gains are not codified.

OPTION 2

Radical change in the existing market

Network orchestrator

CHQ and HHSs jointly set statewide pathways, standards, shared records and a dashboard.

RISK • accountability without authority or funding.

OPTION 3

Expansion into new segments

New markets

Sell education, research partnerships and selected IP beyond Queensland.

RISK • diverts scarce clinicians; returns uncertain.

UNIT OF CHANGE • specific pathways (1) › the whole network's operating model (2) › new external customers (3)

Option 1 is the only option with *no weak test*

CRITERION (WEIGHT)	OPTION 1	OPTION 2	OPTION 3
Suitability 30%	4 	5 	2 
Feasibility 25%	4 	2 	3 
Acceptability: stakeholders 20%	4 	3 	3 
Acceptability: risk 15%	4 	2 	3 
Value / return 10%	3 	4 	2 
Weighted total	3.9	3.3	2.6

ROBUSTNESS • 10,000 RANDOM WEIGHTINGS

97.8%

of weightings rank Option 1 first.

Option 2 wins only if its feasibility reaches *5 out of 5*.

That needs a mandate, shared data and funding — precisely what an Option 1 pilot would help secure.



OPTION 1 • 97.8%

OPTION 2 • 2.2% • OPTION 3 • 0%

Option 2 is the best *long-term* fit; Option 1 is the best *deliverable* fit — and the route to Option 2.

Recommendation: an *ECHO-led, stage-gated* Option 1

Approve Option 1 as CHQ's preferred strategy for 2026–2028: an 18-month, jointly governed pathway program in three pathways.

01 Type 1 diabetes

CENTRAL QUEENSLAND

02 Cleft care

TOWNSVILLE

03 General paediatrics and neurology

WEST MORETON

How — and why — we *modified* Option 1

1

ECHO-led workforce redesign at the core

Targets the capability gap.

2

Joint governance with partner HHSs

Targets the operating-model gap.

3

Funded data foundations and AI assurance

\$0.45m workstream.

4

Stage gates and stop rules

Option 2 decided at month 18; Option 3 deferred.

Value now, with the *option to transform later*: Option 1 is the practical first step to Option 2.

Better access and a protected specialist workforce, for a *small, capped* investment

\$4.4_m

18-month program
0.38% of annual spend

~\$95_m

QCH capital avoided
in a working network

73.7

fewer additional QCH beds
by 2036/37

Value for *every* key stakeholder

Children and families	Less travel, faster follow-up, named coordinator.
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CHQ clinicians	Specialist time on complex care; teaching and career roles.
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Partner HHSs	Local capability; ABF retained locally.
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Department of Health	Service Agreement evidence; capital avoided.
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18 months, four phases, *Board gates* at every step

MONTHS 0-3

Mobilise

Governance group,
MOUs, baselines

MONTHS 3-6

Co-design

Pathway agreements,
safety thresholds

MONTHS 6-15

Pilot

Waves, ECHO, dashboard

MONTHS 15-18

Evaluate and decide

Scale, adapt or stop

GATE • MONTH 6

GATE • MONTH 12

GATE • MONTH 18

What success looks like at *month 18*

Eligible follow-up completed locally

$\geq 50\%$

Median wait, pilot specialties

65 days → -10%

Networked-care KPIs reported

0 of 7 → **7 of 7**

Unplanned escalation and adverse events

No sustained increase

Four *decisions* we ask of the Board

1

Endorse Option 1 for 2026–2028

2

Approve \$4.4m, 18-month program with gates at 6, 12, 18 months

3

Seek Service Agreement recognition of the statewide role

4

Defer Option 2 to a month-18 business case; Option 3 to a six-month asset review

The main risks are *manageable*

Clinical safety	CSCF eligibility, credentialing, CATCH escalation, stop rules.
Specialist workload rises	Fund protected sessions first.
Partner capacity	Backfill, right to decline.
Data gaps	Workstream 8 and data-readiness gate.

PROTECT

Keep complex, high-acuity and genuinely quaternary care *concentrated at QCH*.

EXTEND

Use networked care, ECHO®, shared-care pathways and local capability to bring appropriate care *closer to families*.

BUILD

Use the 18-month stage-gated pilot to establish evidence, governance and data foundations for a *future network-orchestrator* model.

Option 1 delivers value now — while building CHQ's capability to transform later.

OUR ANSWER TO THE OPENING QUESTION

Extend reach by moving *capability*, not
by moving the specialists.

QUESTIONS & DISCUSSION

FOR QUESTIONS

Appendices: method, evidence and *implementation detail*



Research method and *evidence base*

- **Primary sources.** CHQ Annual Report 2024–25, Master Plan 2021/22–2036/37, Strategic Plan 2024–2028, Services Plan 2018–2028, Digital and Data Strategy 2024–2028, Operational Plan FY2025–26
- **Assessment 1 frameworks.** PESTEL, Five Forces and strategic groups, VRIN/VRIO, value chain; CHQ consultation (9 Sep 2026)
- **External evidence.** Peer-reviewed evaluations of paediatric telehealth and Project ECHO; SickKids and GOSH benchmarks
- **Analysis methods.** Weighted SAF scoring; Monte Carlo sensitivity (10,000 Dirichlet draws); Master Plan capacity scenarios; Mendelow stakeholder mapping; risk register
- **Limits.** Scores are team judgement; budget indicative; some studies read from abstracts; refresh KPIs with the 2025–26 Annual Report

Options and core *analytical insights*

	OPTION 1	OPTION 2	OPTION 3
Core idea	Scale advice, outreach, shared care, local follow-up in 2–3 pathways	CHQ and HHSs jointly govern statewide pathways, records, dashboard	Package education, research, IP for external buyers
Builds on	Specialist Online Advice, CATCH, ECHO, Rockhampton and Townsville	Master Plan fully networked scenario; audiology governance model	Paid masterclasses; 805 publications; \$24.16m research funding
Key insight used	Specialist time is the binding constraint; telehealth 125% of target	Root cause is absence of governed statewide system	Expertise is rare and hard to imitate (VRIN)
Data need	Pilot-scale (gap 0.9 pts)	Statewide, near-real-time (gap 2.4 pts)	De-identified data and IP rules first

Detailed evaluation and *decision criteria*

CRITERION	WEIGHT	QUESTION AND RATIONALE
Suitability	30%	Does it fix the specialist bottleneck, growth outside the catchment and the unmeasured network role?
Feasibility	25%	Deliverable in 1–3 years given 14.2% vacancy, near-zero surplus, legacy digital
Acceptability: stakeholders	20%	Families, clinicians, HHSs, Department; CHQ cannot direct HHSs
Acceptability: risk	15%	Clinical, financial, reputational, governance exposure
Value / return	10%	Long-run access, capacity and cost benefit; uncertain for all

SCENARIO	OPT 1	OPT 2	OPT 3
Base case	3.90	3.30	2.60
Suitability-led 45/15/15/10/15	3.85	3.80	2.40
Equal weights	3.80	3.20	2.60
Opt 2 feasibility = 4	3.90	3.80	2.60
Opt 2 feasibility = 5	3.90	4.05	2.60
10,000 random weights	97.8% first	2.2% first	0%

Selected option — *modifications* and rationale

MODIFICATION	WHY	BENEFIT TO CHQ AND STAKEHOLDERS
ECHO-led workforce redesign at the core	Specialist workforce is the binding constraint	Specialists focus on quaternary care; GPs, nurses and allied health gain skills and careers
Joint Pathway Governance Group	CHQ cannot direct other HHSs	Shared accountability; partner buy-in
Workstream 8: data foundations and AI assurance (\$0.45m)	0 of 7 networked-care KPIs reported; data maturity 1.9/5	Measurable network; safe AI; gate for Option 2
Capped budget, stage gates, stop rules	\$0.13m surplus; no financial headroom	Reversible; limits downside; tests \$95m capital case
Option 2 deferred to month 18; Option 3 to asset review	Option 2 lacks mandate and data; Option 3 diverts clinicians	Preserves the option to transform

Evidence for the *ECHO-led tiered* workforce model

ROLE	FUNCTION	QUEENSLAND EVIDENCE
GPs and primary care	Present before referring; manage stable conditions	ADHD ECHO: self-efficacy 2.6 to 5.4 of 7; SC4C trial: referrals 7.3% to 3.0%
Nurse practitioners	Protocol-based triage; chronic condition clinics	9 CHQ subspecialties; 85% of clinicians rate a success
Nurse navigators	Named case manager across services	400-navigator program, gains on all hospital measures
Advanced-practice allied health	First assessment of Cat 2–3 referrals	Sunshine Coast MPSC-MDT: about 96% managed without surgeon
Aboriginal and Torres Strait Islander health practitioners	Community screening and follow-up	Deadly Ears: 563 assessments beside 556 ENT-seen children
GP-led clinic in QCH ED	Low-acuity presentations	About 25% of presentations; ED waits cut 21.6 min

Benefits to CHQ and *key stakeholders*

STAKEHOLDER	RISK OR CONCERN	RESPONSE BUILT INTO THE PLAN
Children and families	Local care seen as a downgrade; delayed escalation	Co-designed consent; family choice; guaranteed rapid CHQ review
CHQ clinicians	Advice on top of full clinics; burnout (68% engagement)	Protected, funded sessions; clinicians co-design pathways
Partner HHSs	Unfunded clinics; liability; recruitment	Signed agreements covering funding, CSCF scope, right to decline
Department of Health	Precedent for a new funding line	Pilot as evidence base for Service Agreement role
Aboriginal and Torres Strait Islander families	Culturally unsafe care; widening gaps	Involve IUIH and community-controlled services; report by population

Detailed *implementation* plan

- **Pilot pathways.** Type 1 diabetes and insulin pumps (Central Queensland); cleft lip and palate follow-up (Townsville); general paediatric and neurology follow-up (West Moreton, Ipswich +3.2% a year)
- **Eight workstreams.** Governance; referral front door; advice and virtual care; shared care; workforce redesign and ECHO; coordination, families and equity; data and evaluation; data foundations and AI assurance
- **Networked Care Office.** A small office in Strategy and Transformation becomes the nucleus of any later orchestrator role
- **Change approach.** Kotter's sequence; early wins from Rockhampton and Townsville

BUDGET LINE	\$M
Protected specialist advice time	1.05
Advanced-practice roles (3.0 FTE)	0.68
Nurse navigators (3.0 FTE)	0.60
Partner HHS backfill and training	0.55
Network data foundations and AI	0.45
Program management and analyst	0.40
ECHO 2.0 expansion	0.35
Telehealth and record configuration	0.15
Independent evaluation	0.15
Total	4.38

Risks, funding safeguards and *data-readiness gate*

RISK	RATING	MITIGATION
Care moves beyond local capability	Medium / High	CSCF eligibility, credentialing, CATCH 24/7, stop rules
Specialist workload rises	High / High	Fund protected sessions first; track minutes per patient
Loss of ABF activity	High / Medium	Telehealth review keeps CHQ revenue; refill freed slots; negotiate advice funding
Data cannot be shared or linked	Medium / High	Data-sharing agreements, privacy impact assessments, The Viewer
Unsafe or biased AI	Low / High	Silent mode first; clinician decides; AI register

OPTION 2 GATE · network data set live in 3+ HHSs at 95%+ audited accuracy; all 7 networked-care KPIs reported; data-sharing agreements signed; shared care plan in routine use; platform funded; AI tools risk-assessed.

Generative AI *acknowledgement*

AI TOOL AND NATURE OF USE

- **Tool.** OpenAI ChatGPT — GPT-5.6 Sol.
- **How it was used.** To support synthesis of the team's CHQ research, presentation structure, concise slide wording, speaker-note drafting and visual organisation.
- **Scope.** Main presentation slides and appendices. AI was used as an assistive drafting and synthesis tool; it was not treated as an authoritative source of CHQ facts.
- **Date.** September 2026.

KEY PROMPTS AND HUMAN OVERSIGHT

- “Consolidate the uploaded CHQ documents into a 12-minute MBA strategy presentation using the final three options and supporting evidence.”
- “Provide speaker notes that justify the strategic analysis, option evaluation, recommendation and implementation plan.”
- “Add relevant appendix slides covering research method, option analysis, evaluation criteria, selected-option rationale, stakeholder benefits and implementation.”
- **Human oversight.** The team reviewed and edited the AI-assisted material, checked factual claims against the cited CHQ and external sources, and retains responsibility for the analysis, strategic judgement and final submission.